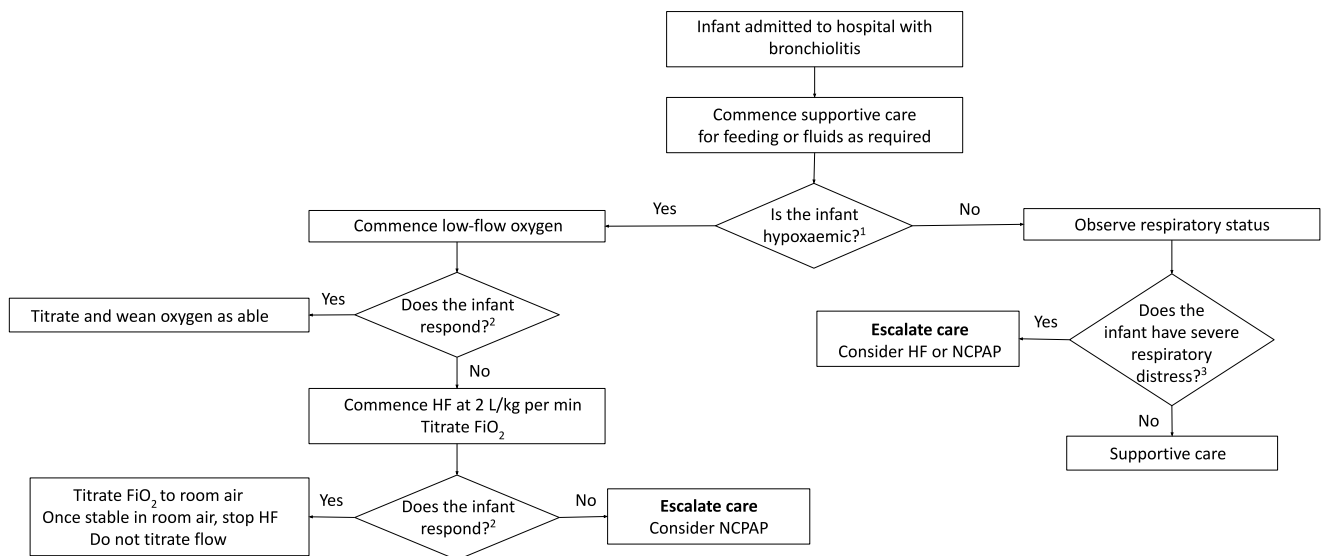


Australasian Bronchiolitis Guideline: 2025 Update

An evidence-based approach to the use of respiratory support in infants with bronchiolitis



FiO₂ = fractional concentration of inspired oxygen; HF = Humidified high flow; nCPAP = nasal continuous positive airway pressure.

¹For otherwise healthy infants aged ≥6 weeks: SpO₂ persistently <90%. For infants aged <6 weeks, or infants <12 months with an underlying health condition: SpO₂ persistently <92%.

²Response to therapy (low-flow or HF oxygen therapy) is determined by a reduction in respiratory rate, a reduction in heart rate, or a paediatric early warning score within 4-5 hours of commencing therapy.

³If at any time, the infant has severe respiratory distress, escalate care. Respiratory distress is a subjective finding. Severe respiratory distress is a level where a senior clinician determines that escalation in care is required, transferring the patient to the emergency department resuscitation area, paediatric ward resuscitation area, high dependency unit, or intensive care unit. Junior staff should escalate concerns regarding severe respiratory distress to senior colleagues.

References

1. Paediatric Research in Emergency Departments International Collaborative (PREDICT). Australasian Bronchiolitis Guideline: 2025 Update. Melbourne, VIC (AU): PREDICT; 2025. Available from: <https://predict.org.au/bronchiolitis-guideline/>